

Risk, Safety & Triage

NCMHCE quick reference · CounselorReady™ — safety is assessed **FIRST**, before every other clinical decision.

Suicide risk — what stacks the acuity

Screen in order: **ideation** → **plan** → **access to means** → **intent** → **protective factors**. A specific plan **with access to means** (or clear intent) is the highest-weight combination. Prior attempt is among the strongest predictors even when current ideation is denied.

Assessment	Response
Structured screen	Administer C-SSRS / Columbia — assess before acting.
Moderate risk, can stay safe	Collaborative safety plan (Stanley-Brown) + means restriction + increase contact.
High risk (plan + means/intent)	Least-restrictive setting that keeps client safe; consider hospitalization; do not leave alone.
Never	No-suicide / no-harm contracts — not evidence-based. Use a safety plan.

Duty to warn / protect (harm to others)

Triggered only by a **specific, imminent** threat to an **identifiable victim**. Then warn/notify per state statute and **document** — disclose the **minimum necessary**. Vague anger → document & monitor.

Level of care — least restrictive that keeps the client safe

Setting	When
Inpatient / crisis	Acute danger or medical instability; 24/7 supervision.
PHP	Safe overnight but needs ~6+ hrs/day structure.
IOP	Needs >weekly structure; still functions at home/work (3–5 days/wk).
Outpatient	Mild–moderate, adequate functioning — the default.

Withdrawal danger ranking (which can kill)

Substance	Danger → action
Alcohol / benzodiazepines	Can be fatal (seizures, DTs). Assess CIWA-Ar ; medically supervised detox. Never advise abrupt cessation.
Opioids	Miserable, rarely fatal. MAT referral; discomfort drives relapse/overdose risk.
Stimulants / cannabis	Primarily psychological; outpatient supportive management.

Ethics & Legal Duties

NCMHCE quick reference · CounselorReady™ — the limits of confidentiality, and the pathways that pierce it.

Three triggers that pierce confidentiality (don't blur them)

Trigger	Pathway
Threat to identifiable person	Duty to warn/protect — specific + imminent + identifiable victim → warn/notify per statute; document.
Suspected abuse (child / elder / dependent adult)	Mandated report on reasonable suspicion — not proof. You report; you do not investigate.
Valid court order	Release minimum necessary . A subpoena ≠ court order — verify validity, consult before releasing.

Informed consent & capacity

Capacity = **understand, appreciate, reason, express a choice**. Unclear capacity + no proxy → **formal capacity evaluation** before consent-dependent treatment. Minors: parent/guardian **consent** + minor **assent**; know state exceptions (some allow independent consent for substance/mental-health/reproductive care).

Boundaries & dual relationships (ACA A.5)

Situation	Rule
Sexual/romantic with current client	Prohibited — no exceptions.
Sexual with former client	Prohibited for 5 years ; even after, burden is on counselor to show no exploitation.
Unavoidable (rural/small community)	Manage , don't reflexively avoid: document, informed consent, boundaries, consultation.

When in doubt

Apply a structured model (**Forester-Miller & Davis**): identify competing principles → **consult** → act within law/policy → **document** contemporaneously. The exam rewards minimum-necessary disclosure + documentation, not “report everything” or “protect confidentiality at all costs.”

Treatment Decision Rules

NCMHCE quick reference · CounselorReady™ — what to do next, for whom, and in what order.

Cross-cutting rules (apply everywhere)

1. Stage of change gates treatment. Precontemplation → **MI only** (never skills/groups/abstinence-as-condition). **2. Stabilize first:** safety, mania, acute withdrawal, medical instability before protocol/trauma/insight work. **3. Sequence beats simultaneity.** **4. Scope:** counselors **refer** for medication and never name specific agents.

Sequencing traps (correct order)

Scenario	Correct first action	The trap
BPD + self-harm	Behavioral chain analysis , then replacement skill	Teaching a skill before the chain analysis
Bipolar depression	Refer for mood stabilization , then add therapy	Intensive therapy / antidepressant monotherapy while cycling
Anorexia, unstable vitals	Medical stabilization first	Starting CBT-E / FBT while medically unstable
PTSD + SUD (not acute w/d)	Integrated/concurrent (COPE)	"Get sober first," delaying trauma work
SUD, precontemplation	Motivational interviewing	Confronting denial; requiring abstinence to treat

Contraindications — when to AVOID

Treatment	Avoid when...
Exposure therapy	Active psychosis, severe dissociation, or acute SI without a safety plan
CBT-I sleep restriction	Bipolar I (mania risk); seizure disorders
Antidepressant monotherapy	Any history of hypomania/mania (screen first — switch risk)
Family-Based Treatment (anorexia)	Active parental abuse; parents can't supervise meals

Assessment Tools by Domain

NCMHCE quick reference · CounselorReady™ — the right instrument, and self-report vs clinician-administered.

Match the tool to the question

Domain	Instrument (S = self-report, C = clinician)	Note
Suicide risk	C-SSRS / Columbia (C)	Assess before any safety plan
Safety planning	Stanley-Brown SPI	After risk is assessed; not a contract
Depression	PHQ-9 (S) · HAM-D (C)	PHQ-9 for monitoring; HAM-D is clinician-rated
Anxiety	GAD-7 (S) · PDSS (C, panic)	Match to the disorder
Bipolar screen	MDQ (S)	Screen before treating depression
Trauma / PTSD	PCL-5 (S) · CAPS-5 (C)	CAPS-5 = gold-standard diagnosis
Substance — screen	AUDIT / CAGE / DAST (S)	Screening, not withdrawal
Substance — withdrawal	CIWA-Ar (C)	Tracks alcohol-withdrawal severity
Cognition	MoCA > MMSE	MoCA more sensitive to mild impairment; rule out delirium first
Eating	SCOFF (S) + vitals/labs	Medical stability before psychosocial
Child / ADHD	Vanderbilt , multi-informant	Cross-setting; developmental history
Foundational	Clinical interview + MSE	Ground assessment before instruments
Cultural context	Cultural Formulation Interview	Avoid over-/under-pathologizing

Defense Mechanisms & Cognitive Distortions

NCMHCE quick reference · CounselorReady™ — name the mechanism, spot the distortion.

Defense mechanisms (mature → immature)

Mechanism	Example
Mature: sublimation	Channeling aggression into competitive sport
Mature: humor / altruism	Using humor to cope; helping others to manage one's own pain
Neurotic: repression	Unconsciously keeping distressing memories out of awareness
Neurotic: displacement	Yelling at family after a conflict with the boss
Neurotic: reaction formation	Being overly kind to someone you resent
Neurotic: intellectualization	Discussing a diagnosis in clinical terms to avoid the feeling
Immature: projection	Attributing your own anger to someone else
Immature: denial	Refusing to accept a terminal diagnosis
Immature: splitting	Seeing others as all-good or all-bad (common in BPD)
Immature: acting out	Expressing distress through behavior instead of words

Cognitive distortions (+ how to restructure)

Distortion	Example → restructure
All-or-nothing	"If it's not perfect, I failed" → look for the middle ground
Catastrophizing	"This will be a total disaster" → examine the actual evidence/odds
Overgeneralization	"This always happens to me" → name the specific instance
Mind reading	"They think I'm incompetent" → check the evidence; ask
Emotional reasoning	"I feel worthless, so I am" → separate feeling from fact
Should statements	"I should always cope" → replace with preference, not demand
Personalization	"It's my fault they're upset" → map the shared causes